

ELECTRONIC HEALTH RECORDS
AN INTRODUCTION TO CODING AND BILLING
May 2025

This document shows two additional coding examples.

CASE 1: INPATIENT CONSULTATION

PATIENT:

72-year-old male with pneumonia and new onset atrial fibrillation

REASON FOR CONSULTATION:

Evaluation and management of new onset atrial fibrillation in patient admitted for pneumonia.

HISTORY OF PRESENT ILLNESS:

72-year-old male admitted 2 days ago for community-acquired pneumonia. During hospitalization, developed irregular pulse and palpitations yesterday. EKG confirmed new onset atrial fibrillation with rapid ventricular response (rate 142). No prior history of arrhythmias. Denies chest pain but reports mild shortness of breath.

PAST MEDICAL HISTORY:

Hypertension, hyperlipidemia, COPD, gout

MEDICATIONS:

Home: Lisinopril, simvastatin, tiotropium inhaler, allopurinol

Hospital: Ceftriaxone, azithromycin, albuterol/ipratropium nebulizer

ALLERGIES: NKDA

REVIEW OF SYSTEMS:

Comprehensive 14-point ROS obtained and documented in EMR

Positive for: Fever, cough with yellow sputum, shortness of breath, palpitations

Negative for: Chest pain, syncope, hemoptysis, nausea, vomiting

PHYSICAL EXAMINATION:

VS: T 99.8°F, BP 152/94, HR 138 (irregular), RR 24, O2 sat 92% on 2L NC

General: Elderly male in mild respiratory distress

HEENT: Normocephalic, atraumatic, oropharynx clear

Neck: JVP elevated 6 cm, no bruits

Cardiovascular: Irregularly irregular rhythm, tachycardic, no murmurs

Respiratory: Crackles in right middle and lower lung fields

Abdomen: Soft, non-tender, non-distended

Extremities: Trace bilateral lower extremity edema

DIAGNOSTIC STUDIES:

Labs: WBC 14,200, improved from admission. BNP 450 pg/mL

EKG: Atrial fibrillation with RVR, rate 142, no ST-T wave changes

CXR: Right middle and lower lobe infiltrates, improved from admission

Echocardiogram: EF 55%, normal LV size and function, mild LA enlargement

ASSESSMENT:

1. New onset atrial fibrillation with rapid ventricular response (I48.91)
2. Community-acquired pneumonia, right middle and lower lobes (J18.9)
3. Essential hypertension (I10)
4. COPD (J44.9)

PLAN:

1. Rate control with metoprolol 5mg IV now, then 25mg PO BID
2. Anticoagulation with heparin drip, transitioning to apixaban 5mg BID
3. Continue antibiotics for pneumonia
4. Cardiology follow-up after discharge
5. Discussed risks/benefits of rhythm vs. rate control strategies
6. Continue respiratory therapy for COPD management

MEDICAL DECISION MAKING:

High complexity due to acute illness with systemic symptoms, prescription drug management requiring monitoring, and management of multiple chronic illnesses.

APPLICABLE CODES:

- ICD-10: I48.91 (Unspecified atrial fibrillation), J18.9 (Pneumonia, unspecified organism), I10 (Essential hypertension), J44.9 (COPD, unspecified)
- CPT: 99253 (Inpatient consultation, detailed history and examination with high complexity medical decision making)

CASE 2: EMERGENCY DEPARTMENT VISIT

PATIENT INFORMATION:

68-year-old female with acute abdominal pain

CHIEF COMPLAINT:

Severe abdominal pain for 12 hours

HISTORY OF PRESENT ILLNESS:

68-year-old female presents with severe RLQ abdominal pain that began gradually yesterday but has progressively worsened. Pain is 8/10, sharp, and radiating to the right flank. Associated with nausea, vomiting x2, and fever. Last bowel movement yesterday, normal. Denies diarrhea, constipation, or blood in stool. No prior similar episodes.

PAST MEDICAL HISTORY:

Hypertension, hyperlipidemia, osteoarthritis

MEDICATIONS:

Lisinopril, atorvastatin, acetaminophen prn

ALLERGIES:

Penicillin (rash)

REVIEW OF SYSTEMS:

Constitutional: Fever, malaise

Cardiovascular: Denies chest pain, palpitations

Respiratory: Denies shortness of breath, cough

GI: As noted above

GU: Denies dysuria, frequency

PHYSICAL EXAMINATION:

VS: T 101.2°F, BP 146/92, HR 104, RR 18, O2 sat 97% RA

General: Elderly female in moderate distress

HEENT: Normocephalic, atraumatic, mucous membranes dry

Cardiovascular: Tachycardic, regular rhythm, no murmurs

Respiratory: Clear to auscultation bilaterally

Abdomen: Tender to palpation in RLQ with guarding and rebound tenderness. Positive Murphy's sign. No distension.

GU: Deferred

Extremities: No edema

DIAGNOSTIC STUDIES:

Labs: WBC 15,800 with left shift, Hgb 13.4, Plt 220K

Chemistry: BUN 25, Cr 1.1, remainder within normal limits

Urinalysis: Negative for infection

CT Abdomen/Pelvis: 8mm calculus at gallbladder neck with surrounding inflammation, gallbladder wall thickening, and pericholecystic fluid consistent with acute calculous cholecystitis

MEDICAL DECISION MAKING:

High complexity with extensive testing and consultation with surgery for management of acute condition

ASSESSMENT:

1. Acute calculous cholecystitis (K80.00)
2. Essential hypertension (I10)

PLAN:

1. Admission to surgical service
2. NPO
3. IV fluid hydration
4. IV antibiotics (Ceftriaxone and Metronidazole)
5. Pain management with IV morphine
6. Surgical consultation obtained, recommending laparoscopic cholecystectomy
7. Consent obtained for surgery

APPLICABLE CODES:

- ICD-10: K80.00 (Acute calculous cholecystitis), I10 (Essential hypertension)
- CPT: 99285 (Emergency department visit, high complexity)
- The documentation supports the 99285 code based on comprehensive history and examination with high complexity medical decision making.